

*Example 12:* [...] Some researchers argue against the use of germline editing for therapeutic uses that there may never be a time when genome editing in embryos will offer a benefit greater than that of existing technologies, such as preimplantation genetic diagnosis (PGD) and in-vitro fertilization (IVF). [...]

However, scientists and bioethicists acknowledge that in some cases, germline editing can address needs not met by PGD. This includes when both prospective parents are homozygous for a disease-causing variant (they both have two copies of the variant, so all of their children would be expected to have the disease); cases of polygenic disorders, which are influenced by more than one gene; and for families who object to some elements of the PGD process. (NHGRI 2017)

